INTUBATION CHECKLIST

Please record: 1) Saturations at end of bagging, 2) Lowest saturations during 1st intubation attempt, and 3) Time between (1) and (2)

Plan

- Is the environment optimal?
- Do I need more help?
- Previous difficult intubation/mask ventilation?

Equipment

- ECG, BP cuff (2 minute cycle), saturations, end tidal CO_2
- Bag valve mask, suction (Yankauer), ETT (half size up and down), laryngoscope and blades, ETT tapes, McGill's forceps, oropharyngeal airway, Stylet/Bougie
- Drugs - RSI, rescue drugs (CONSIDER fluid boluses, atropine, adrenaline); check iv line patency

Patient

- Pre-oxygenate - 4 minutes in 100% oxygen
- CONSIDER nasal cannulae for apnoeic oxygenation
- Optimise patient's position
- Inotropes required?
- Aspirate naso/orogastric tube
- Draw up maintenance sedation infusion

Roles

- Role allocation - Team Leader, Airway Assistant (able to give cricoid pressure if needed?), Doctor in charge of drugs, Nurse in charge of drugs, Scribe

Time-out

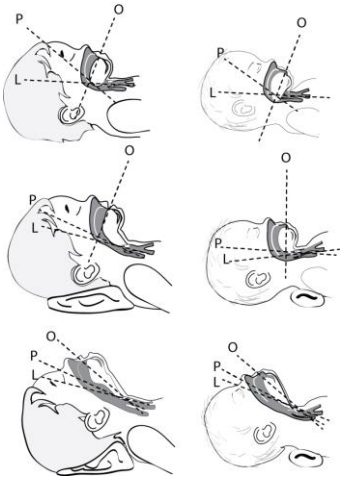
- Briefing to whole team: Any specific complications expected, contingency plan in case of difficulty
- Does anyone have any questions or concerns?
- Record saturation trend and timing (see box at top of page)

Please refer to Guideline in CICU Handbook 'RSI in Children' (Nov 2015)

INTUBATION CHECKLIST

Optimise patient's position

- **Neck flexion/extension as appropriate**
  - Aim to get oral, pharyngeal and laryngeal axes in line for optimal view
  - Child: aim for 'sniffing the morning air' position, i.e. flexion at the cervical spine and extension of the atlantoaxial joint. Consider placing pillow under the child's head to achieve this.
  - Neonates and infants (due to prominent occiputs) as well as patients with unstable cervical spine and Trisomy 21, consider aiming for a neutral head position instead. Consider placing a small towel under the infant's shoulder to achieve this.
- **Head position**
- **Height of bed**



Child | Infant

P: Pharyngeal axis

O: Oral axis

L: Laryngeal axis

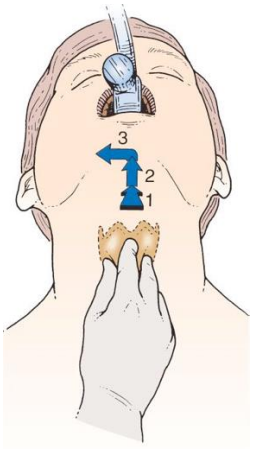
Cricoid Pressure:

B - Backwards

U - Upwards

R - Rightwards

P - Pressure



Apnoeic Oxygenation Flow Rates (standard nasal cannulae):

<1yr: 5L/min

1-10yr: 8L/min

>10yr: 12L/min

*Please refer to Guideline in CICU Handbook 'RSI in Children' (Nov 2015)